

Me and my girlfriends sat up one night drinking a handle of vodka from the liquor cabinet in this one girl's house. We put it in orange juice and 7UP—and it tasted great! It was the first time I ever had fun. We laughed and ran around the house and acted crazy. It was a blast. I was sicker than a dog the next day, which was awful, but I knew I'd do it again as soon as I could.

Therapist: Uh-huh. It sounds like you were able to forget everything and just be wild kids, is that right? (*Ida nods.*) And it was unusual for you to have fun with friends.

Ida's Drinking Part: Oh yeah, my dad was so sick. He was bedridden my whole childhood with one thing or another. Since my mother hated taking care of him, it was up to us. We came right home from school and got him to the bathroom. I knew he was waiting. It was kinda terrible.

Therapist: Mm-hmm. So much pressure on Ida to be responsible. So you were the part who helped her get a little down time, be a little lighthearted, and act like a kid.

Ida's Drinking Part: I guess I was! Otherwise, it was just depressing. So hopeless. Nothing to look forward to—like ever.

Therapist: Right, so do you think there were some even younger or more tender parts who felt hopeless about anything ever getting better?

Ida's Exile: I'm still hopeless.

Ida is now blended with the hurting, hopeless exile, along with the drinking part.

Therapist: Right, yes of course. You've felt hopeless for a long time. It's like it never goes away. We're going to help you. But I wonder if you could be patient for just a little longer. We need permission from the drinking part first. Is that okay?

Ida's Exile: Okay.

Therapist: Great. Thank you. So now I want to talk with the drinking part again. (*Ida nods.*) I wonder, as the part of Ida who drinks, if you're aware that drinking keeps that hopelessness quiet?

Ida's Drinking Part: Well, I know if she didn't drink, she'd be miserable.

Therapist: Right. You've been protecting Ida from feeling hopeless and miserable. I know it would be tough and scary to stop. Your help has been essential! But now if Ida could help that hopeless part so it felt stronger and not so alone, would you get a break?

Ida's Drinking Part: I guess so. But I'm not sure anyone can help.

Therapist: Right, you'd need evidence of that. Ida and I would need to show you. And, with your permission, we will.



This session informed Ida about the benign intentions of her drinking part and opened her heart to its feelings. It also introduced her to a key exile. While it is unconventional to invite a client to come to—or stay in—a session while engaging in their compulsive behavior, it can serve to reassure parts quickly and powerfully that we mean what we say: All parts are welcome. We won't judge.

WHEN FIREFIGHTERS RUN THE SYSTEM

Keep in mind that the line between a firefighter and a manager gets blurred when the client's functioning is very compromised at the far end of the addictive continuum. At this extreme, the addictive process becomes a proactive, rather than a reactive, activity. The craving for pleasure or escape gives way to the need to maintain the habit or to avoid feeling sick. The client now must find a way to engage in the compulsive activity—such as locating money to score drugs or pay for their next sexual encounter—in order to properly function. To do so, firefighters assume the proactive role of managing the client's days—hour by hour, minute by minute. They can be impressively and disturbingly resourceful about lying, stealing, and dealing. As consequences pile up, including those that are physical, emotional, legal, and relational in nature, they just work harder to avoid the obstacles—anything to engage in the activity.

When the client is in this circumstance, we need to be more proactive as well in terms of helping them find a safe environment. If we feel we need to advocate for more intensive treatment, like a residential inpatient program, we are honest and transparent with the client about this. We acknowledge up front that many parts are not interested in listening to our message. We speak to abandoned, fragile parts about being cared for and safe, and we let manager parts know that external containment measures will give them a break.

When we take this route, protective firefighter parts can rant aggressively and contemptuously, accusing us of abandonment or betrayal. In this case, we will probably need to help our own managers stay calm so we can guide the client to hear from the parts inside who do want to survive—who yearn for health, safety, and freedom from compulsion. We may want to tell the client that we believe they have the internal resources to feel whole, steady, and at peace. As the client considers intensive treatment options, we are a supportive resource and we stay available (if possible) as the client completes the inpatient stay or intensive treatment program. Here is an example.

Lukas, a 34-year-old, Lithuanian American, single, gay man, had recently left his job in retail and returned to live with his mother in her home. She set Lukas up in therapy, reporting deep concerns about his daily drinking, drug use, and angry outbursts, which she called dangerous. At the time of this session, the therapist had been seeing Lukas for a few months. He acknowledged his mother's concerns and identified some manager parts who agreed with her and others who didn't like living with her, which meant he needed to find another job. These parts were polarized with the firefighter parts who loved using and didn't think it was all so bad. They said his mother drank a lot as well and shouldn't judge him.

Lukas said he started using in high school, attended local college sporadically, and felt okay with working in retail when he still had the job; he left his previous position after he was upset over a relationship ending badly. All his friends drank and took pills regularly, and he said he couldn't imagine not using. He had recently shown up late and missed a few appointments, and this week, he reported having an intense fight with his mother after she accused him of stealing. He had been sleeping on a friend's couch.

WORKING WITH A PROACTIVE FIREFIGHTER



Therapist: You look pretty exhausted.

Lukas's Firefighter: I am. I just got home, and my mother went crazy on me because I was gone for a couple of days. It's none of her business what I do. She's nuts anyway!

Therapist: She called me and left a message saying you took money from her. That sounds pretty intense and miserable. If you took money from her, she doesn't have much.

Lukas's Firefighter: She called you?! What the hell, I didn't steal from her! She's always on my case—

Therapist: (*Therapist interrupts.*) She sounded pretty clear. Can you tell me more about what happened the last few days?

Lukas's Firefighter: My friends and I got fucked up, what else do you want to know?

Therapist: Your drinking and drugging parts took over for a few days. And it sounds like another part of you took money from your mother to make that happen. (*Lukas sputters.*) Lukas, you don't have any income these days. It sounds like your using parts got desperate. So why did you come home?

The therapist invites the other parts to speak up.

Lukas's Firefighter: I didn't take money from my mother! (*The therapist doesn't respond.*) I don't know why I came home. I wanted to get out of there. I guess I needed sleep.

Therapist: Okay. And you made it here to your appointment. So can I reflect back what I'm hearing? (*Lukas shrugs.*) Your using parts ran the show for a few days. To serve their agenda, another part who, based on what you've been showing me, has amped up lately, took some money from her. (*Lukas opens his mouth to protest again.*) Let me finish! And it will not admit to doing that because other parts of you are dismayed.

Lukas's Firefighter: Whatever.

Therapist: This binge has affected your body. You're exhausted. And it has affected your mood. You're screaming your head off with your mother, which has also been happening more—

Lukas's Firefighter: I told you, she's a drunk. Where does she get off calling you?! She just wants me to be home with her—

Therapist: (*Therapist interrupts gently.*) Yep, your mom has some real issues of her own, no question. Let's leave her out of it for now, okay? I'm more interested in your perspective.

Lukas's Firefighter: My perspective is you should leave it alone, too. I'm fine. I don't need this shit from you, either.

Therapist: I got it. I know your partying parts couldn't care less what anyone else thinks or feels. I also get that in their own way, they help you. You like being with friends, and when you're high, you feel better. I get that. And I also see you are here, in my office, at your appointment. I know that some of your parts have said they want more in life than just using. Your partying parts are taking more risk now. They will do just about anything. But I also see other parts in action. You've also tried to help your mom sometimes. And you're a good friend. And you've got parts who have been through tough times. No dad, kids bullying you, being in the closet. I know you've been through a lot. So, I'm wondering if there are also maybe one or two small parts, who feel, even briefly, pretty worn out, worried, or just tired of the craziness. Like you could use a break?

Lukas's Firefighter: I could use a break from all this bullshit!

Therapist: I hear your angry part who wants to defend you. I understand that a lot of your parts do not want to listen to any of this. I also know that you really wanted that relationship to work out, and you were heartbroken. You've had to be strong on numerous occasions. I know your partying parts give you a break. And I know there is more to you than just using and getting fucked up. Speaking for my drug counselor parts, I'm just going to focus on the using right now and say I am worried. I know you're frustrated with your mother, but she does care about you. The stakes are getting higher, and I see parts getting involved in stuff that you don't even want to admit, like stealing. I'm sure I don't know all of it, either. In my experience, those parts make sure you have the means to get high but, in the end, you feel even worse about yourself.

Lukas's Firefighter: I am fine, you don't need to worry about me. This is bullshit! I'm leaving.

Therapist: Of course, you can leave. Since my parts are worried about you, I'm texting you the name of the treatment center we talked about, just so you have it. I know the people there. They have a couple of different programs that you could check out. Like I said, the stakes are getting high for you and you're taking bigger risks now. I care about you, and I'm not alone. I understand that many of your parts don't want to stop using, which is what they would want you to do at the treatment center. And I know that you have other parts who want a break. It doesn't have to be this hard, Lukas. There are other ways to go. You can text me. I am happy to keep meeting with you, or you can text the center, where you could get a break for a while. *(Lukas walks out, slamming the door.)*



Lukas continued to use heavily for a few more weeks, occasionally calling or texting the therapist but not showing up for an appointment. When she got him on the phone for a few minutes, she continued to listen to his parts and reflected that some of them needed a different life, while some wanted to keep using. She also asserted that his using parts were trying to help him and wondered if they didn't need a break too. A month later, Lukas was arrested for shoplifting. He was offered the option of substance use treatment and went to a local center. After that, he entered a halfway house, looked for work, and returned to his IFS therapist.

After returning to therapy, Lukas and his therapist interviewed his using parts, and they identified these parts' clear desire to take him away from or medicate his abandoned, wounded exiles. He was curious about recurrences and discovered that his using parts had a message for him about exiles who needed attention. He learned that his using parts wished he would be more autonomous when his caretaking, compliant managers worked to avoid conflict with his mother or with certain friends. He also got connected to an LGBTQ+ Alcoholics Anonymous (AA) group, which gave him a supportive new social community.

By listening to every one of his using parts, Lukas was able to create his own vision for the future. Some of his parts were in alignment with the AA commitment to sobriety from alcohol, but he did have other parts who still wanted to smoke cannabis and relax with friends occasionally. Because of his occasional cannabis use, no one would sponsor him, but he did discover other AA friends who were not wedded to AA's clean and sober mantra and who were experimenting with their own version of a productive and balanced life.

EXILE INTERVENTIONS

Whenever clients focus internally, they can get flooded with the warded off memories and feelings of exiles, which is called an *exile jailbreak*. In response, firefighters will activate instantly. They may distract (for example, through dissociation), or they may start hunting for some way of soothing (*I want chocolate!*). Meanwhile, managers will get anxious and start to criticize. Our job, as Schwartz

says, is to be the eye of the storm. Accordingly, we ask protectors to let us reassure the exile and negotiate with it to be patient.

For example, Irina, a thirty-five-year-old, Slovenian American, partnered, gender-neutral person, first came to therapy for posttraumatic stress disorder and chronic restrictive eating patterns. In past sessions, most of their